



Darul-ilm Kent

Safeguarding & Child Protection Policy

Academic year	2026 – 2027
Date adopted	
Review date	August 2026 (annual)
Policy owner	Mohammed Didar Hasan, Founder & Head Administrator
Designated Safeguarding Lead (DSL)	Abdul-Lateef Agiri, Teacher: 07446788597
Deputy DSL	Hanif Uddin, Administrator: 07368446235
Students served	Ages 5 – 18 (weekday and weekend sessions)
Admin contact	07778020745

1. Policy Statement

Darul-ilm Kent (“the Madrasa”) is committed to safeguarding and promoting the welfare of every child and young person who attends our weekday and weekend classes. We believe that the protection of children is the responsibility of every member of our community, teachers, administrators, volunteers, trustees, and parents alike.

Islam teaches us that children are a trust (amānah) placed in our care. The Prophet ﷺ showed extraordinary kindness and patience towards children. Anas ibn Mālik (RA) served him for ten years and reported he was never once rebuked. Ibn ‘Umar (RA) reported him saying: “All of you are shepherds and every one of you is responsible for his herd.” This responsibility is the foundation of our commitment to safeguarding. Any harm or abuse of children goes directly against the teachings of Islam and will not be tolerated at this Madrasa.

This policy applies to all staff, volunteers, and visitors in contact with children at Darul-ilm Kent, whether on premises or in any other setting including online delivery.

We believe that:

- The welfare of the child is paramount, above all other considerations.

- All children, regardless of age, gender, disability, race, religion, or background, have an equal right to protection from abuse and harm.
- All concerns and allegations will be taken seriously, recorded, and referred to the appropriate authorities without delay.
- **Cultural or community pressure, including concern about shame or reputation, must never prevent a safeguarding concern from being reported. The child's safety always comes first.**
- Physical punishment of any kind is strictly forbidden and constitutes abuse.
- Safeguarding is everyone's responsibility, not only the DSL's.
- Staff must maintain an attitude of 'it could happen here', abuse occurs in faith settings and no community is immune.

2. Scope

This policy applies to all teaching staff (employed and self-employed contractors), administrative staff, volunteers, trustees, and any visitor on Madrasa premises. It covers all weekday evening sessions, weekend morning sessions, off-site trips, and online or remote teaching.

3. Legal Framework & Guidance

- The Children Act 1989 and 2004
- Working Together to Safeguard Children 2026 (HM Government)
- Keeping Children Safe in Education 2024 (DfE), applied as best practice for supplementary settings
- The Safeguarding Vulnerable Groups Act 2006
- The Counter-Terrorism and Security Act 2015 (Prevent duty)
- The Serious Crime Act 2015 (FGM mandatory reporting)
- The Muslim Council of Britain, Safeguarding Children at Mosques and Madrassahs (2023)
- Kent Safeguarding Children Multi-Agency Partnership (KSCMP) procedures

4. Roles & Responsibilities

4.1 Designated Safeguarding Lead (DSL)

The DSL and deputy DSL must be known to all staff, volunteers, parents, and students. Their names and contact details must be displayed visibly on Madrasa premises at all times.

The DSL is responsible for:

- Receiving, recording, and acting on all safeguarding concerns and disclosures
- Referring to Medway Children's Social Care (MASH), 01634 334466, where there is concern about abuse or neglect of a child at home or in the community (e.g. disclosure of abuse by a family member, neglect, domestic violence in the household, a parent collecting a child while intoxicated)

- Referring to the Local Authority Designated Officer (LADO), Medway LADO: 01634 331043, where an allegation has been made against a member of staff, volunteer, trustee, or any adult working with children at the Madrasa
- Contacting the police (999) where a child is in immediate danger or where a criminal offence may have been committed (e.g. sexual assault, FGM, forced marriage)
- Using the NSPCC helpline (0808 800 5000) for confidential advice when uncertain whether a concern meets the threshold for a formal referral
- Ensuring all staff complete safeguarding training before starting work and annually thereafter
- Maintaining a confidential safeguarding log, stored securely and separately from other records
- Reviewing this policy annually and updating it when guidance changes
- Ensuring safeguarding is a standing agenda item at all management meetings
- Reporting serious incidents to the Charity Commission where required

Designated Safeguarding Lead	Abdul-Lateef Agiri, Teacher: 07446788597
Role	Teacher
Contact	07446788597
DSL training completed	
Deputy DSL	Hanif Uddin
Deputy contact	07368446235

4.2 All Staff & Volunteers

Every person working with children at the Madrasa, whether paid or unpaid, must:

- Be alert to signs of abuse, neglect, or concerning behaviour, maintain an attitude of 'it could happen here'
- Never promise confidentiality to a child who discloses abuse
- Report any concern to the DSL the same day, without delay
- Never investigate a concern themselves or confront an alleged perpetrator
- Complete safeguarding training as required and keep it up to date
- Adhere to the staff code of conduct at all times
- Challenge and report any behaviour by colleagues that gives cause for concern (whistleblowing)

4.3 Head Administrator / Founder

- This policy is implemented, displayed, and reviewed annually
- A DSL and deputy DSL are appointed, trained, and their details are publicly displayed
- All staff and volunteers have valid enhanced DBS checks before working with children
- A Single Central Record (SCR) of all checks is maintained securely
- Safeguarding is a regular agenda item at management meetings
- Parents and students are made aware of who the safeguarding leads are

4.4 Trustees

Trustees bear ultimate responsibility for ensuring safeguarding processes are in place and followed. They must ensure the policy is implemented, the DSL role is properly resourced, and serious incidents are reported to the Charity Commission.

5. Recognising Abuse & Harm

All staff must be familiar with the following forms of abuse and harm. No single indicator confirms abuse, but any concern, however small, must be reported to the DSL. It is the DSL's role to decide what action to take, not the individual member of staff's.

5.1 The Four Core Categories of Abuse

Physical Abuse	Intentionally causing physical harm to a child, including hitting with hands or objects, slapping, punching, kicking, shaking, burning, scalding, biting, throwing, or breaking bones. Physical punishment by staff is abuse and is strictly prohibited at this Madrasa.
Emotional Abuse	The ongoing emotional mistreatment of a child. This includes deliberately trying to scare, humiliate, isolate, or ignore a child, making them feel worthless, unloved, or frightened. Witnessing domestic abuse is itself emotional abuse.
Sexual Abuse	Forcing or enticing a child to take part in sexual activities, whether or not the child appears to understand or consent. Contact abuse involves physical contact with the child. Non-contact abuse includes involving children in sexual images, online grooming, or exposure to sexual material. Sexual abuse can be carried out by adults or by other children, and by women as well as men.
Neglect	The persistent failure to meet a child's basic physical and/or psychological needs, likely to cause serious harm. The most common form of child abuse. A child may be left hungry, dirty, without clothing, shelter, supervision, or medical care.

5.2 Additional Forms of Harm

Child Sexual Exploitation (CSE) & Grooming

CSE is a form of sexual abuse in which a child is manipulated or coerced into sexual activity, often in exchange for gifts, money, affection, status, drugs, or alcohol. Children are frequently tricked into believing they are in a genuine relationship, this is called grooming. The child may not realise they are being abused. Perpetrators can be of any age, gender, or background, including people known and trusted by the family.

Grooming can take place over weeks or years, in person or online. Groomers may build relationships with the child's family or friends to appear trustworthy or authoritative.

Factors that can increase a child's risk of CSE

- Being unsure about, or unable to disclose, their sexual orientation to family
- Lacking friends from the same age group; social isolation
- A history of abuse, risk of forced marriage, or honour-based violence
- Living away from home, in residential care, a hostel, B&B, or supported accommodation
- Low self-esteem or self-confidence
- Attending school or being friends with other young people who are being sexually exploited

Additional behavioural signs of CSE

- Having nightmares or disturbed sleep
- Substance or drug misuse
- Suddenly having unexplained money, gifts, or possessions
- Not being allowed to have friends, particularly in adolescence
- Saying they have secrets they cannot tell anyone about
- Spending time with much older people or in unfamiliar locations

⚠ Community Pressure & Cultural Shame, A Critical Warning

Cases of sexual abuse in madrasah settings have been documented across the UK. In some instances, children were abused over extended periods because adults in the community prioritised cultural shame or the reputation of a family over the safety of the child. In one documented IICSA case, a child was abused at a madrasah from age 8 to 11, and when she came forward, community members harassed her rather than supporting her.

Darul-ilm Kent is clear: concern about shame, community reputation, or the family of an alleged perpetrator must NEVER influence whether a safeguarding concern is reported. Any person who withholds, suppresses, or discourages a report for these reasons will have committed a serious breach of this policy.

Peer-on-Peer (Child-on-Child) Abuse

Abuse is not always carried out by adults. Children can and do abuse other children, including physical abuse, bullying, sexual harassment, sexual assault, and online abuse. All such reports must be taken seriously. Both the alleged victim and alleged perpetrator require appropriate support. The DSL must be informed immediately.

Bullying & Cyberbullying

Bullying is repeated behaviour that hurts someone, including name-calling, hitting, spreading rumours, threatening, and undermining. Cyberbullying occurs through messaging apps, social media, gaming platforms, or email. All reports must be taken seriously and recorded.

Online Abuse

Online abuse is any form of abuse facilitated through internet-connected devices. It can occur via social media, messaging apps, email, gaming platforms, and live-streaming. Children can be at risk from people they know as well as strangers. Online abuse may run alongside offline grooming or bullying. Staff must not use personal accounts to communicate with students.

Sexting

Sexting refers to young people sending or receiving sexually explicit images, videos, or messages, usually via smartphone or social media. Where the person depicted is under 18, this constitutes an indecent image of a child and is a criminal offence to create, possess, or share, even if the young person sends an image of themselves, and even if both parties consider it harmless. Sexting can lead to serious and lasting harm, including blackmail, bullying, and significant emotional distress, particularly where an image is shared more widely than intended.

Staff who become aware that a student has sent, received, or is being pressured to send such an image must report this to the DSL immediately. Staff must never view, forward, or store such an image themselves under any circumstances, doing so is itself a criminal offence. The DSL will determine the appropriate response, which may include referral to the police or children's social care.

Age of Consent

In England and Wales, the legal age of consent for any sexual activity is 16, regardless of gender or sexual orientation. It is a criminal offence for anyone to engage in sexual activity with a person under 16. The law provides additional specific protection for children aged 12 and under, who cannot legally consent to any sexual activity under any circumstances. Where a member of staff or volunteer (aged 18 or over) is in a position of trust, any sexual activity with a person under 18 in their care is treated as an abuse of that position of trust, regardless of any apparent consent.

Female Genital Mutilation (FGM)

FGM is when a girl's genitals are deliberately altered or removed for non-medical reasons. It is illegal in the UK and constitutes child abuse. There is no basis in the Qur'an or Sunnah for FGM, it has no Islamic justification whatsoever. Staff who discover or suspect that FGM has been carried out on a child under 18 are legally required to report it to the police. This is a mandatory duty and is not subject to professional judgement.

Warning signs that a child may be at risk of FGM before it occurs can include: knowledge that the family belongs to a community where FGM is practised, knowledge that an older sibling or the mother has undergone FGM, the family making preparations for the child to travel abroad, arranging vaccinations ahead of travel, or planning an unexplained absence from school or the Madrasa. A child may also talk about a 'special procedure' or ceremony that is going to take place, or about being prepared for marriage.

Forced Marriage & Honour-Based Abuse

A forced marriage is one where one or both parties do not or cannot give free and full consent. Honour-based abuse refers to violence, threats, or coercion used to protect the

perceived honour of a family or community. Both are criminal offences. Report immediately to the DSL. The Forced Marriage Unit: 020 7008 0151.

Child Trafficking & Criminal Exploitation

Child trafficking involves children being moved and then exploited, forced to work, or sold. County Lines is a specific form in which children are used to transport drugs across county boundaries, children as young as 12 have been exploited in this way. Signs include unexplained absences, new older 'friends', unexplained money or gifts, and changes in behaviour. Report any concern to the DSL.

Domestic Abuse

Domestic abuse includes controlling, bullying, threatening, or violent behaviour between people in a relationship. Witnessing domestic abuse is in itself child abuse and can cause lasting harm to a child's development and mental health. It can happen in any household regardless of background.

Radicalisation & Extremism (Prevent)

Radicalisation is the process through which a person comes to support extremist ideologies, which may lead to terrorism. As a faith community, we are vigilant against any ideology, including violent interpretations of religion, being introduced to young people. Concerns must be reported to the DSL who will consider a referral to the Channel programme.

Non-Recent (Historical) Abuse

A child or adult may disclose abuse that occurred in the past. This must be taken just as seriously as a current concern and reported to the DSL in the same way. Historical allegations against staff or volunteers must be referred to the LADO.

6. Detailed Indicators of Abuse

The following indicators are intended to help staff recognise possible abuse. No single indicator is conclusive. Staff should report any concern to the DSL without trying to investigate or reach a conclusion themselves.

6.1 Physical Abuse, Indicators

Signs in the child, physical

- Bruising in or around the mouth, cheeks, ears, or neck
- Two simultaneous bruised eyes without bruising to the forehead (rarely accidental)
- Repeated or multiple bruising on the head, back, stomach, chest, or genital/rectal areas
- Bruising or injuries of varying ages, suggesting repeated harm over time
- Marks in the shape of an object: belt marks, hand prints, cord marks
- Linear bruising, particularly on the buttocks, back, or face
- Bruising or tears around or behind the earlobes (caused by pulling or twisting)
- Grasp marks on the upper arms, forearms, or legs

- Burns or scalds with a clear outline (cigarettes, hot objects, immersion in hot water)
- Scalds to the buttocks or feet, a child cannot accidentally scald their bottom without scalding their feet
- Fractures with vague or inconsistent explanation, or associated with old fractures
- Bite marks (those over 3cm in diameter are more likely caused by an adult or older child)
- Unexplained injuries or injuries inconsistent with the explanation given

Signs in the child, behavioural / emotional

- Refusal to discuss injuries
- Claiming punishment was excessive
- Fear of parents being contacted, or fear of going home
- Withdrawal from physical contact, flinching when approached
- Keeping arms and legs covered in warm weather
- Fear of seeking medical help
- Aggression towards peers
- Frequent or unexplained absences

Signs in the parent or family environment

- Not seeking medical help, or unexplained delay in seeking treatment
- Reluctant to give information or mention previous injuries
- Absent when their child is brought for treatment
- Disinterested or undisturbed when their child has been injured
- Aggressive towards the child or others
- History of domestic violence, mental health issues, or substance misuse
- Past history of childhood abuse in the family

Note: Physical punishment by staff is abuse regardless of severity or religious justification. Any use of physical force on a student must be reported to the DSL immediately.

6.2 Emotional Abuse, Indicators

Signs in the child, behavioural / emotional

- Developmental delay without physical cause
- Frozen watchfulness, particularly in younger children, appearing unnaturally still and alert
- Low self-esteem and lack of confidence
- Withdrawn or isolated; difficulty relating to others
- Over-reaction to mistakes; extreme fear of getting things wrong
- Fear of new situations
- Neurotic behaviour: rocking, hair twisting, thumb sucking
- Self-harm

- Extremes of passivity or aggression
- Depression and withdrawal
- Chronic running away or compulsive stealing
- Air of detachment: 'I don't care' attitude
- Poor peer relationships
- Attention-seeking or hyperactive behaviour

Signs in the parent or family environment

- Scapegoats one child in the family
- Conveys to the child that they are worthless, unloved, or inadequate
- Deliberately silences the child or mocks what they say
- Imposes developmentally inappropriate expectations on the child
- Abnormal attachment to the child (overly anxious or completely disinterested)
- History of domestic violence, mental health issues, or substance misuse

6.3 Neglect, Indicators

Signs in the child, physical

- Failure to thrive; underweight or short stature without medical cause
- Frequent hunger, asking for or stealing food
- Dirty or unkempt appearance; clothing in poor repair or inappropriate for the weather
- Dry or sparse hair; pale or mottled skin
- Recurrent or untreated infections, skin conditions (severe eczema, persistent head lice, nappy rash), or dental decay
- Unmanaged medical conditions
- Frequent accidents or injuries suggesting inadequate supervision
- Constant tiredness

Signs in the child, behavioural / emotional

- General developmental delay, especially speech and language
- Inadequate social skills
- Attachment disorders; absent normal social responsiveness
- Emotionally needy or indiscriminate with adults
- Compulsive stealing
- Frequently absent or late
- Poor self-esteem and destructive tendencies
- Thrives away from home environment
- Self-harming behaviour

Signs in the parent or family environment

- Failure to meet the child's basic needs for food, clothing, warmth, and hygiene
- Failure to attend medical appointments or seek appropriate treatment
- Child left with adults who are intoxicated or violent
- Child abandoned or left alone for excessive periods
- Dangerous or hazardous home environment
- History of neglect, mental health, alcohol or drug misuse, or domestic violence in the family

6.4 Sexual Abuse, Indicators

Signs in the child, physical

- Urinary infections, bleeding, or soreness in the genital or anal areas
- Recurrent pain on passing urine or faeces
- Blood on underclothes
- Sexually transmitted infections
- Vaginal soreness or bleeding
- Injuries to the genital or anal area, or bruising to the buttocks, abdomen, or thighs
- Pregnancy in a young girl where the identity of the father is undisclosed or surrounded by secrecy

Signs in the child, behavioural / emotional

- Makes a direct disclosure
- Demonstrates sexual knowledge or behaviour inappropriate to their age, unusually explicit
- Draws sexually explicit pictures
- Inexplicable changes in behaviour: becoming aggressive or very withdrawn
- Self-harm, eating disorders, or self-mutilation
- Poor self-image and self-hatred
- Running away from home
- Poor concentration, appearing to be 'in a world of their own'
- Sudden changes in academic performance or truancy
- Inappropriate sexualised conduct towards peers or adults
- Wetting or other regressive behaviours (thumb sucking, baby talk) in older children
- Depression or extreme anxiety
- Reluctance to be alone with a particular adult
- Fear of undressing (e.g. for wudu or PE)

Signs in the parent or family environment

- Lack of appropriate sexual boundaries
- Grooming behaviour towards the child or other children

- Comments suggesting inappropriate knowledge of or involvement in the child's sexual development
- Wider vulnerabilities: domestic violence, substance misuse, mental health difficulties

Remember: sexual abuse can be carried out by any person regardless of age, gender, or relationship to the child. A trusted teacher, family friend, or older student can be a perpetrator. Do not let familiarity or community standing affect your judgement.

6.5 Children with Disabilities, Additional Considerations

Children with disabilities may be at heightened risk of abuse and may find it harder to disclose. Additional indicators to be aware of include:

- A bruise in a site that might be unremarkable on a walking child (e.g. the shin) may be significant on a non-mobile child
- Not receiving adequate help with feeding, leading to malnourishment
- Poor toileting arrangements or lack of personal care
- Lack of stimulation
- Unjustified or excessive use of restraint
- Rough handling or extreme behaviour management
- Unwillingness to learn or accommodate a child's means of communication
- Ill-fitting equipment or inappropriate physical arrangements

Staff working with children with disabilities or SEN should be especially alert and should discuss any concern with the DSL promptly.

7. Reporting Procedures

7.1 The Four Rs

All staff should remember the four Rs when a concern arises:

Step	Action	What this means in practice
Recognise	Identify signs of abuse or harm	Use the indicators in Section 6. Trust your instinct, if something feels wrong, report it.
Respond	Act without delay	Listen calmly if a child discloses. Do not investigate. Go to the DSL the same day.
Record	Write it down accurately	Use the child's own words. Note time, date, location, what was said or seen. Give to DSL.
Refer	Pass to the right person or agency	DSL refers to children's social care, police, or other agencies as appropriate.

7.2 When a Child Makes a Disclosure

If a child tells you something that suggests they are being abused or are at risk:

- Stay calm and listen carefully. Do not show shock, disbelief, or alarm.
- Let the child speak at their own pace. Do not interrupt, prompt, or ask leading questions.
- Tell the child you are glad they told you and that you will do your best to help.
- Do NOT promise confidentiality or that you will keep what they have said secret.
- Do NOT investigate the concern yourself or confront the alleged perpetrator.
- Do NOT ask the child to remove or adjust clothing to show you injuries.
- If the concern relates to sexual abuse, it is especially important not to probe, children may be frightened of the implications of 'telling' or may be under threat of reprisals.
- As soon as possible, write down exactly what was said using the child's own words. Note time, date, location, who was present, and any observed non-verbal behaviour or visible injuries.
- Report to the DSL immediately, the same day. Pass your written record in a sealed envelope.
- Take no further action after reporting to the DSL, all decisions on next steps rest with the DSL.

You may tell the child about confidential helplines: Childline 0800 1111 / NSPCC 0808 800 5000.

7.3 Reporting to the DSL

All concerns, disclosure, observation, or third-party report, must reach the DSL the same day. The DSL will record the concern, assess the risk, decide whether to refer to external agencies, and inform the Head Administrator (unless the concern involves them).

7.4 Referral to External Agencies

Medway Children's Social Care (MASH)	01634 334466
Emergency / immediate danger	999
NSPCC Helpline (free, 24/7)	0808 800 5000
Childline	0800 1111
Prevent / Channel referral	0800 011 3764 or local police
LADO, allegation against staff	Medway LADO: 01634 331043
Forced Marriage Unit	020 7008 0151
FGM, mandatory reporting	999 (police) or local social care
Child trafficking / Modern Slavery	08000 121 700

Do NOT delay reporting

A concern must never go unreported because of uncertainty about whether it 'counts', or because you hope the situation will resolve. If in doubt, report to the DSL. Doing nothing is not an option.

8. Allegations Against Staff or Volunteers

Inappropriate behaviour by staff can take many forms, including physical (use of force as punishment), emotional (intimidation, belittling, sarcasm, discrimination), sexual (sexualised behaviour, harassment, or assault towards students), and neglect (failing to protect or act on a concern). Although it is an uncomfortable thought, it must be acknowledged that abuse can occur in madrasah settings.

If an allegation is made against any member of staff, volunteer, or trustee:

- The Head Administrator must be informed immediately (unless the allegation is against them, in which case the most senior trustee is informed).
- The Local Authority Designated Officer (LADO) must be contacted within one working day. The Madrasa will not conduct its own investigation before consulting the LADO.
- The member of staff or volunteer may be suspended pending investigation. Suspension is neutral and does not imply guilt.
- All allegations will be treated seriously, documented, and handled confidentially.
- The Charity Commission must be notified of serious incidents.
- The alleged victim must be supported throughout the process.
- If the Head Teacher / Administrator is the subject of the allegation, the Chair of Trustees takes on the liaison role with the LADO.

Staff who are concerned about a colleague's conduct must report this to the DSL or Head Administrator immediately. This is a whistleblowing duty and staff are protected from detriment for raising genuine concerns in good faith.

9. Safer Recruitment

Before any person starts working at the Madrasa (paid or voluntary), the following checks must be completed and recorded:

- Enhanced DBS check with children's barred list check
- Verification of identity (passport or birth certificate and photo ID)
- Proof of right to work in the UK
- We may ask for at least two references, one from a previous role involving children where possible
- Confirmation of relevant qualifications
- Completion of a thorough induction including this policy and the code of conduct

No person may have unsupervised access to children before their DBS check is returned clear. DBS checks must be renewed every three years or when there is cause for concern. A Single Central Record (SCR) of all checks will be maintained by the Head Administrator and audited annually.

The fact that someone is known to the community, is a practising Muslim, or has taught elsewhere does not remove the requirement for full checks. Safe recruitment is itself a safeguarding measure.

10. Code of Conduct for Staff & Volunteers

- Treat all students with dignity, respect, and kindness at all times
- Never be alone with a single child in a closed room, doors should remain open or glass panels must be visible
- Never use physical punishment, shout aggressively, or use demeaning or frightening language
- Never share personal contact details with students or communicate via personal social media or messaging apps
- Never share or view inappropriate images involving children
- Never transport a child alone in a private vehicle without explicit written parental consent
- Never give a student gifts, money, or personal items without the knowledge of the Head Administrator and parents
- Immediately disclose to the DSL any personal circumstances (e.g. a police investigation or conviction) that may affect suitability to work with children
- Report any colleague's behaviour that causes concern, however senior that person may be

Breach of this code is a serious disciplinary matter and may result in suspension, termination, and referral to the DBS or police.

11. Online Safety

- Online lessons must use a platform approved by the Head Administrator. Sessions should not use personal accounts.
- Teachers must not communicate with students via personal WhatsApp, social media, or email. All contact must be through official Madrasa channels.
- Parents must be informed of all online teaching arrangements and provide consent.
- Any online safeguarding concern, including grooming, inappropriate content, or cyberbullying, must be reported to the DSL immediately.
- Online abuse is as serious as in-person abuse and will be treated as such.

12. Prevent Duty & Radicalisation

The Madrasa takes seriously its duty to protect students from radicalisation under the Counter-Terrorism and Security Act 2015. Staff must be alert to signs including:

- Identity or personal crisis, distanced from cultural/religious heritage, sense of isolation or low self-esteem
- Expression of extreme or violent views, or glorification of terrorism
- Accessing extremist content online or possessing extremist literature or symbols associated with an extremist cause
- Significant changes in behaviour, friendship groups, or appearance
- Spending increasing time in the company of other suspected extremists, with a loss of interest in friends and activities unrelated to the cause
- Justifying the use of violence to address grievances, or condoning hate, violence, or harm towards others
- Attempting to recruit others to a group, cause, or ideology
- Identifying another group as a threat and blaming that group for social or political problems, including the use of insulting or derogatory language towards them
- Seeking to join extremist organisations

Any such concern must be reported to the DSL, who will consider a Channel referral. Promoting Islamic values of mercy, justice, and community cohesion is both a religious and safeguarding responsibility.

12A. Self-Harm

Self-harm describes a range of behaviours in which a young person deliberately causes themselves harm, often as a response to overwhelming emotional pain, anger, distress, fear, or low self-esteem. It can include cutting, burning, biting, hitting, hair pulling, picking or scratching at the skin, substance misuse, or restrictive or excessive eating or exercise. Self-harm can become a way for a young person to cope when they feel they have no other option, and can be addictive.

If a student self-harms while at the Madrasa, the immediate priority is appropriate first aid, followed by informing the DSL. The DSL will consider whether the underlying cause is itself a safeguarding concern, for example abuse, bullying, or exploitation, and will respond accordingly, including referral to external agencies where appropriate.

Staff must never suggest or encourage any technique that involves causing physical discomfort, pain, or shock as a way of coping with the urge to self-harm. Such methods are not considered safe or appropriate by current safeguarding and mental health guidance. The right response is to listen without judgement, avoid criticising the student, and ensure the matter reaches the DSL so the underlying distress can be properly addressed, including, where needed, signposting to a GP, CAMHS, or another appropriate professional source of support.

Where a student is known to self-harm, the DSL should consider with relevant staff whether any additional support or a simple risk-awareness plan is needed, always handled sensitively and confidentially.

13. Safe Physical Environment

- Premises are secure during sessions, entry and exit points are monitored via the rota duty system
- Children are dismissed into the care of a parent or a known adult. If an unfamiliar adult, or anyone the child does not recognise, comes to collect a child, staff will check with the parent before releasing the child. Younger children in particular are not released to anyone the staff or child cannot confirm.
- Any unknown adult attempting to collect a child must be questioned. If unsatisfactory, the child must not be released and the DSL or police contacted.
- Children must not be left unsupervised on premises at any time
- Male and female students are appropriately separated in line with Islamic principles and safeguarding best practice
- Only DBS-checked, authorised staff should have unsupervised access to children on site
- First aid provision is maintained on site. A list of trained first aiders is displayed on the premises.
- Fire safety procedures are in place. A list of fire marshals is displayed on the premises.
- Safeguarding posters naming the DSL and deputy and how to contact them are displayed prominently throughout the building

13.1 Mixing of Age Groups

Darul-ilm Kent teaches students aged 5 to 18 across weekday and weekend sessions. Where younger and older students are required to mix, for example during arrival, departure, communal events, or award ceremonies, this must only happen where necessary and under direct adult supervision at all times.

Class groupings should keep age ranges as narrow as reasonably possible. Where older students (e.g. 16–18) are present at the same time as much younger children (e.g. 5–8), staff must remain alert to the wider age gap and ensure younger children are not left in the unsupervised company of older students. Any pattern of an older student forming an unusually close or exclusive relationship with a much younger child must be raised with the DSL.

Staff should be especially attentive to vulnerable children and should discuss any concern promptly with the DSL. If a vulnerable child's attendance changes unexpectedly or they stop attending, the DSL must be informed immediately. Vulnerable students may include:

- Children with disabilities or special educational needs
- Children living with domestic violence
- Children with mental health difficulties
- Children with a history of abuse or neglect
- Children living in poverty or with inadequate parental supervision
- Children who are isolated socially or from their family

Staff should be especially attentive to vulnerable children and should discuss any concern promptly with the DSL. If a vulnerable child's attendance changes unexpectedly or they stop attending, the DSL must be informed immediately.

14. Confidentiality & Record Keeping

All safeguarding concerns and records are confidential and shared only with those who need to know to protect the child. The child's safety always takes precedence over confidentiality.

Safeguarding records are kept securely and separately from general records. Individual files are kept per child, not per family. Records are retained for a minimum of 25 years (or until the child's 25th birthday if later). If a child moves to another setting, the DSL must transfer their safeguarding file directly to the receiving DSL, not via the child or parents.

A written record must be kept of who has accessed safeguarding records and when.

15. Training

All staff and volunteers must complete a recognised safeguarding training course (minimum Level 1) before starting work and annually thereafter, ideally at the start of each academic year. The DSL must hold enhanced DSL-level training, refreshed every two years.

All staff must sign to confirm they have received, read, and understood this policy at induction and whenever it is updated. Free training is available through:

- Medway Council Children's Services education team
- The Education People (Kent County Council's education partner)
- NSPCC Learning, learning.nspcc.org.uk
- Thirtyone:eight (specialist faith community safeguarding), thirtyoneeight.org, 0303 003 1111

16. Policy Review

This policy will be reviewed annually in August, or sooner if legislation changes, a safeguarding incident occurs, or a DSL / Head Administrator changes. All staff will be notified of updates and asked to re-sign.

Policy Sign-Off

Position	Name
Founder & Head Administrator	Mohammed Didar Hasan
Date adopted _____	Review date Annually, August each year
Signature _____	

Appendix A: Key Contacts at a Glance

These contacts must also be displayed visibly on Madrasa premises.

Designated Safeguarding Lead	Abdul-Lateef Agiri, Teacher: 07446788597
Deputy DSL	Hanif Uddin, Administrator: 07368446235
Head Administrator	07778020745
Medway Children's Social Care (MASH)	01634 334466
Medway LADO	01634 331043
Police (emergency)	999
Police (non-emergency)	101
NSPCC Helpline (free, 24/7)	0808 800 5000
Childline	0800 1111
Prevent / Channel	0800 011 3764
Forced Marriage Unit	020 7008 0151
Modern Slavery Helpline	08000 121 700
Thirtyone:eight (faith safeguarding)	0303 003 1111

Appendix B: Staff Safeguarding Agreement

To be signed by all staff and volunteers at induction and each time the policy is updated.

Full name	
Role	
I confirm that I have read and understood the Darul-ilm Kent Safeguarding & Child Protection Policy	
I understand that the welfare of children is paramount and that it is my duty to report any concern to the DSL without delay	
I understand that cultural or community pressure must never prevent me from reporting a concern	
I agree to abide by the code of conduct set out in this policy	
Signature	
Date	

Appendix C: Safeguarding Concern Record Form

Complete as soon as possible after the incident. Pass to the DSL in a sealed envelope immediately. Do not investigate yourself. Your notes may be needed in court proceedings, accuracy is essential.

Full name of child	
Date of birth / age	
Date & time of concern	
Location	
Name of person recording	
Role	
Nature of concern (circle)	Disclosure by child / Observation / Third-party report / Other

Details, use the child's own words where a disclosure was made. Do not interpret or assume.	
Any physical signs observed	
Were any injuries visible? If yes, describe and indicate location on a body diagram if possible.	
Who else was present?	
Has this happened before or been reported before?	
Action taken immediately	
DSL informed, name, date & time	
Referred to external agency?	Yes, agency: _____ Date: _____ No, reason: _____
Was the child told of next steps?	
Signature & date	

Appendix D: Safeguarding Induction Checklist for New Staff

To be completed by the Head Administrator or DSL at induction. A signed copy to be kept on the staff member's file.

Staff name	
Role	
Date of joining	
Date of induction	
Induction conducted by	
DBS check reference & date	
Safeguarding policy given and explained	Yes / No
Code of conduct explained and signed	Yes / No

DSL and deputy DSL introduced / identified	Yes / No
Reporting procedure explained	Yes / No
Online safety expectations explained	Yes / No
Prevent duty explained	Yes / No
Safeguarding training completed or booked	Yes / No, Date: _____
Staff signature DSL / Head Administrator signature	